

Wrightington, Wigan and Leigh: why the headline hides the pressure

An investigation into the major A&E at Wigan Infirmary, using NHS England's monthly A&E statistics (January 2021 – September 2026).

Prepared by Pius Ajamma · 9 October 2026 · Independent research, not commissioned by or affiliated with the NHS.
New to the terms? See the glossary in Appendix C.

47.3%

of patients in WWL's major A&E seen within 4 hours in 2023-24, against a trust headline of 68.7%

1.8×

England's rate of 12-hour waits for a bed, 2023-24 to 2025-26

–43%

fall in WWL's 12-hour bed waits, Apr–Sep 2026 vs Apr–Sep 2025

–4.3 pts

change in the major A&E's 4-hour score over the same period

Summary

WWL's headline 4-hour score is lifted by its urgent treatment centres, which see about 99% of patients within 4 hours. Its major A&E, the Emergency Department at the Royal Albert Edward Infirmary in Wigan, has seen only about half of its patients within 4 hours since 2022-23.

- **It is a WWL-specific problem, not only a national one.** Until 2021-22 WWL's major A&E tracked the North West. From 2023-24 it ran 5–9 points below the North West and 8–12 points below England every year.
- **It is not caused by more arrivals.** Major A&E visits fell by about 14% between 2021-22 and 2023-24, yet performance worsened.
- **Bed access was the clearest pressure point.** From 2023-24 to 2025-26, WWL patients were almost twice as likely as the English average to wait over 12 hours for a bed after a decision to admit, which points to patient flow and bed capacity at Wigan Infirmary.
- **There is genuine recent improvement, and a remaining problem.** Comparing the same months a year apart, 12-hour bed waits fell by 43% while England's rate rose. But the major A&E's 4-hour score fell further, which suggests delays now sit with patients who are not admitted.
- **The data cannot confirm the cause.** Admitted vs non-admitted breakdowns and time-to-assessment data would be needed to test the explanations in section 6.

1. What triggered the investigation

The investigation started in the dashboard's Trust League Table, which ranks trusts with a major A&E by their 4-hour performance. For 2023-24, WWL sat mid-table on its headline score, close to its neighbours. But its major A&E (Type 1) score was far lower than theirs.

Trust	Region	Type 1 Attendances	4hr %	Type 1 4hr %	vs Target	12hr Wait
FOUNDATION TRUST						
LIVERPOOL UNIVERSITY HOSPITALS NHS FOUNDATION TRUST	North West	207,267	68.0%	56.5%	-8.0%	14,118
JAMES PAGET UNIVERSITY HOSPITALS NHS FOUNDATION TRUST	East of England	81,383	68.1%	62.3%	-7.9%	3,115
MID AND SOUTH ESSEX NHS FOUNDATION TRUST	East of England	373,761	68.1%	66.1%	-7.9%	236
BARTS HEALTH NHS TRUST	London	287,186	68.4%	55.7%	-7.6%	9,062
PORTSMOUTH HOSPITALS UNIVERSITY NHS TRUST	South East	128,979	68.5%	56.3%	-7.5%	227
UNIVERSITY HOSPITALS DORSET NHS FOUNDATION TRUST	South West	160,771	68.5%	60.2%	-7.5%	1,425
FRIMLEY HEALTH NHS FOUNDATION TRUST	South East	262,565	68.6%	65.5%	-7.4%	753
WRIGHTINGTON, WIGAN AND LEIGH TEACHING HOSPITALS NHS FOUNDATION TRUST	North West	88,072	68.7%	47.3%	-7.3%	4,151
KINGSTON AND RICHMOND NHS FOUNDATION TRUST	London	84,495	68.8%	50.4%	-7.2%	3,985
MERSEY AND WEST LANCASHIRE TEACHING HOSPITALS NHS TRUST	North West	190,740	68.8%	57.7%	-7.2%	2,879
Total		16,525,835	69.7%	58.9%	-6.3%	439,411

Figure 1. Trust League Table, 2023-24, sorted by headline 4-hour score. WWL's row shows 68.7% overall but 47.3% in its major A&E. Screenshot from the Power BI report.

Trusts with almost the same headline score in 2023-24:

Trust	4hr % (all types)	Type 1 4hr %	Gap (points)
Mid and South Essex	68.1%	66.1%	2.0
Liverpool University Hospitals	68.0%	56.5%	11.5
Barts Health	68.4%	55.7%	12.7
Wrightington, Wigan and Leigh	68.7%	47.3%	21.4

Three signals justified a closer look: two related measures disagreed sharply; WWL was an outlier against trusts with the same headline; and a stakeholder ranking trusts by the headline alone would have placed WWL mid-table and moved on.

2. What sits behind WWL's numbers

Before analysing, the services counted in WWL's figures were mapped. A text search of the organisation list for "Wigan", "Leigh" and "Wrightington" returned a single organisation, WWL (code RRF). There are no separately reported providers in the area. WWL itself runs both kinds of service:

Service	Location	Type in the data	Typical 4hr %
Emergency Department (24/7)	Royal Albert Edward Infirmary (Wigan Infirmary)	Type 1: major A&E	47–53%
Wigan Urgent Treatment Centre	Royal Albert Edward Infirmary (same site)	Other: urgent care	~99%
Leigh Urgent Treatment Centre	Leigh Health Centre	Other: urgent care	~99%

Leigh Infirmary and Wrightington Hospital are also part of WWL but have no A&E: Wrightington specialises in planned orthopaedic surgery, which is outside this dataset.

Split of WWL's visits, 2023-24:

	Visits	Share
Major A&E (Type 1)	88,072	59%
Urgent treatment centres (Other)	62,092	41%
All visits	150,164	100%

Because about 4 in 10 visits go to the fast urgent treatment centres, they pull the trust's average well above its major A&E's score. The data reports figures per trust, not per building, so all findings below refer to WWL as a trust.

3. Is it one bad year or a pattern?

WWL's figures for each NHS financial year (April to March):

NHS year	Major A&E visits	UTC visits	Overall 4hr %	Major A&E 4hr %	UTC 4hr %	12hr bed waits
2020-21*	21,332	7,153	88.6%	84.8%	99.9%	68
2021-22	102,319	54,833	76.5%	64.1%	99.6%	583
2022-23	96,464	56,010	68.6%	50.9%	99.1%	2,427
2023-24	88,072	62,092	68.7%	47.3%	99.0%	4,151
2024-25	85,408	63,104	70.6%	49.7%	98.9%	4,960
2025-26	88,094	62,346	71.9%	53.0%	98.6%	5,107
2026-27*	40,745	37,070	73.7%	50.9%	98.8%	1,293

* Partial years: 2020-21 covers January–March 2021 (a period of suppressed demand during COVID); 2026-27 covers April–September 2026, with no winter months yet.

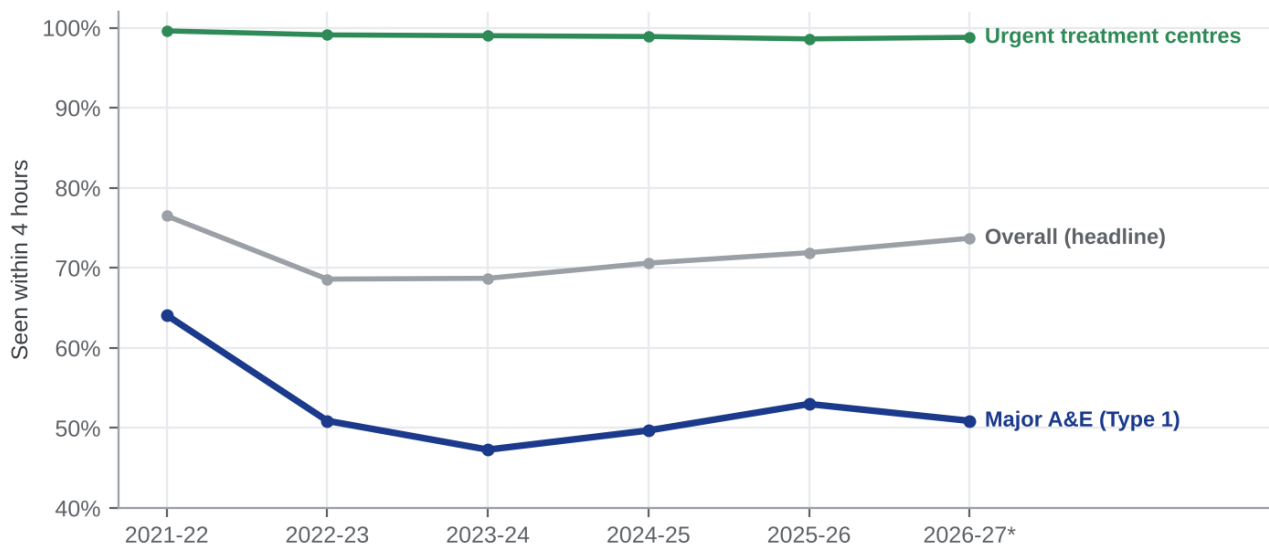


Figure 2. WWL's three 4-hour scores. The headline sits between a near-perfect urgent care score and a major A&E score of about 50%.

What this shows

- **A persistent pattern, not one bad year:** the major A&E fell from 64.1% to 47.3% over two years and has only recovered to about 50%.
- **Fewer arrivals, worse performance:** major A&E visits fell from 102,319 to 88,072 while the score dropped, so volume does not explain it.
- **Bed waits grew ninefold:** 12-hour waits from decision to admit rose from 583 to 5,107 a year.
- **A mix effect flatters the headline:** the urgent treatment centres' share of visits rose from 35% (2021-22) to 48% (2026-27 to date), lifting the overall score without the major A&E improving.

4. Is WWL worse than comparable hospitals?

A score of 50% only means something against a benchmark. WWL's major A&E was compared with all major A&Es in the North West and in England. Bed waits are shown per 1,000 major A&E visits, so trusts of different sizes can be compared fairly.

NHS year	WWL major A&E	North West	England	WWL vs England	WWL 12hr waits per 1,000	England 12hr waits per 1,000
2020-21*	84.8%	74.0%	79.1%	+5.7	3.2	1.8
2021-22	64.1%	61.5%	70.4%	-6.3	5.7	6.1
2022-23	50.9%	53.5%	62.0%	-11.1	25.2	25.3
2023-24	47.3%	56.7%	58.9%	-11.6	47.1	26.6
2024-25	49.7%	56.4%	59.3%	-9.6	58.1	31.8
2025-26	53.0%	57.7%	60.9%	-7.9	58.0	33.3
2026-27*	50.9%	59.3%	61.7%	-10.8	31.7	32.7

* Partial years, as in section 3. Highlighted rows: WWL's 12-hour wait rate was about 1.8 times England's.

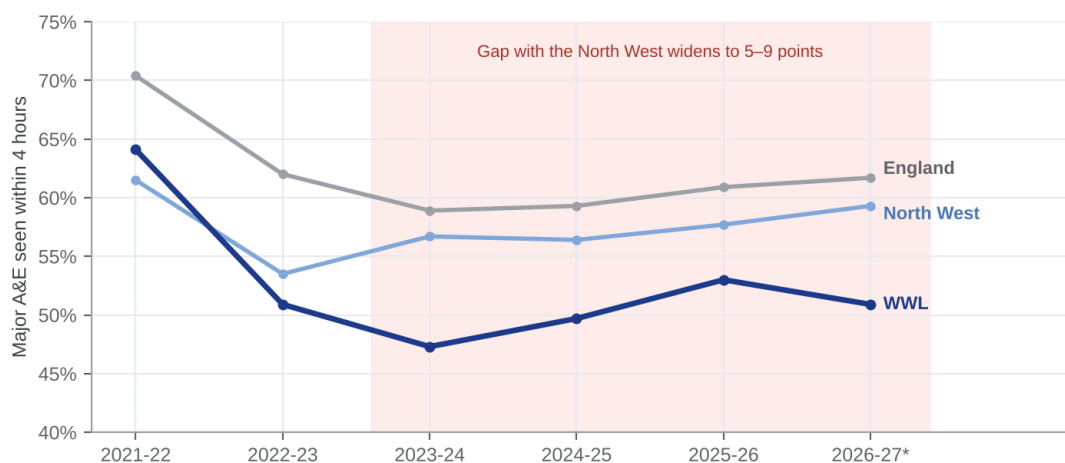


Figure 3. Major A&E (Type 1) 4-hour performance: WWL vs North West vs England.

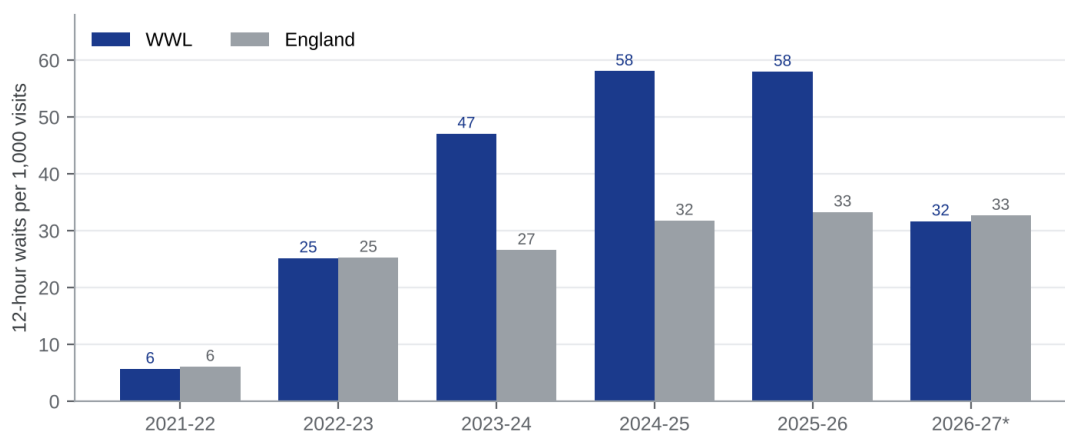


Figure 4. 12-hour waits from decision to admit, per 1,000 major A&E visits.

What this shows

- **Part of the problem is regional:** the whole North West performs below England.
- **Part is specific to WWL:** from 2023-24 WWL ran 5-9 points below the North West every year.

- **Bed access diverged in 2023-24:** WWL's 12-hour wait rate matched England exactly in 2022-23 (25.2 vs 25.3), then rose to nearly double the national rate for three years.

5. Is 2026-27 a real improvement? A like-for-like test

2026-27 so far looked much better on bed waits, but it contains no winter months. To remove the seasonal effect, April–September 2026 was compared with April–September 2025.

Measure	Apr–Sep 2025	Apr–Sep 2026	Change
WWL 12-hour bed waits	2,260	1,293	–43%
WWL 12-hour waits per 1,000 visits	52.2	31.7	–39%
England 12-hour waits per 1,000 visits	28.4	32.7	+15%
WWL major A&E 4-hour %	55.2%	50.9%	–4.3 points
England major A&E 4-hour %	61.6%	61.7%	+0.1 points

Two results, both important.

Bed waits genuinely improved. Because the months are identical, the season cannot explain it, and it happened while the national rate got worse. Admitted patients at WWL are reaching a bed faster.

The 4-hour score got worse. This can happen alongside the first result because the two measures count different patients: 12-hour waits count only admitted patients, while the 4-hour score counts everyone in the major A&E, including those sent home.

6. What it means

Conclusions supported by the data

- WWL's headline 4-hour score materially overstates the performance of its major A&E, and the gap has widened as more patients use its urgent treatment centres.
- The major A&E's under-performance is persistent (four years) and, from 2023-24, worse than both the North West and England.
- The deterioration was not driven by rising arrivals. The sharp rise in 12-hour bed waits from 2023-24 points to patient flow out of A&E (often called exit block) as a key factor in that period.
- Between the same months of 2025 and 2026, bed waits improved substantially against the national trend, but the 4-hour score did not.

Possible explanations for the remaining 4-hour gap (not tested)

- **Case mix:** with more minor cases streamed to the urgent treatment centres, the Emergency Department is left with sicker, slower patients.
- **Delays for non-admitted patients:** assessment, diagnostics or senior review times inside the Emergency Department.
- **Recording or streaming changes** between the Emergency Department and the urgent treatment centre, which share the Wigan Infirmary site.

Limitations

- Figures are reported per trust, so results cannot be attributed to individual hospital buildings.
- The dataset has no split between admitted and non-admitted patients and no time-to-assessment data.
- The 12-hour wait rate uses major A&E visits as its denominator, as an approximation of exposure.
- Partial years (2020-21 and 2026-27) are flagged and excluded from like-for-like conclusions.
- The urgent treatment centres' consistent ~99% may partly reflect how activity is recorded.

Recommended next steps

- Add NHS England's admitted vs non-admitted 4-hour breakdown to test where the remaining delay sits.
- Repeat the like-for-like test after winter 2026-27 (October 2026 – March 2027) to see whether the bed-wait improvement holds under winter pressure.
- Benchmark WWL against peer trusts of similar size and case mix, not only regional and national totals.
- Report Type 1 performance alongside the headline score for every trust, so averages cannot hide pressure in major A&Es.

Appendix A. Data, method and validation

Data

NHS England, *Monthly A&E Attendances and Emergency Admissions*, trust-level CSVs, January 2021 – September 2026 (69 months). Contains public sector information licensed under the Open Government Licence v3.0.

Pipeline and model

- A Python script downloads every monthly file; a second script cleans, combines and reconciles them into a star schema (fact_ae, dim_org, dim_date) loaded into Power BI.
- Data quality issues handled: national TOTAL rows typed inconsistently (and once stored in the organisation columns), a source link missing "A&E", empty junk columns in one file.
- All KPIs are DAX measures in the Power BI semantic model; the analysis queries below run against it.

Validation

Check	Result
Reconciliation to NHS England TOTAL rows	All 69 months match, zero variance
4-hour performance, all types (Sep 2026)	74.4%, matches NHS England's published figure
Type 1 4-hour performance (Sep 2026)	60.6%, matches published figure
Total emergency admissions (Aug 2026)	517,868, matches published figure
Cross-page consistency	12hr waits 2023-24 = 439,411 on both report pages

Key DAX queries

Benchmark query (section 4):

```
EVALUATE
SUMMARIZECOLUMNS (
    dim_date[nhs_fy],
    "WWL big A&E %", CALCULATE ( [Type 1 4hr Performance %], dim_org[org_code] = "RRF" ),
    "North West big A&E %", CALCULATE ( [Type 1 4hr Performance %], dim_org[region_short] = "North West" ),
    "England big A&E %", [Type 1 4hr Performance %],
    "WWL 12hr waits per 1,000 visits",
        CALCULATE ( DIVIDE ( [12hr Waits from Decision to Admit], [Type 1 Attendances] ) * 1000,
            dim_org[org_code] = "RRF" ),
    "England 12hr waits per 1,000 visits",
        DIVIDE ( [12hr Waits from Decision to Admit], [Type 1 Attendances] ) * 1000
)
ORDER BY dim_date[nhs_fy]
```

Core measure (validated at 60.6% against NHS England for September 2026):

```
Type 1 4hr Performance % =
DIVIDE ( [Type 1 Attendances] - [Type 1 Over 4 Hours], [Type 1 Attendances] )

Type 1 Attendances = SUM ( fact_ae[att_type1] ) + SUM ( fact_ae[att_booked_type1] )
Type 1 Over 4 Hours = SUM ( fact_ae[over4h_type1] ) + SUM ( fact_ae[over4h_booked_type1] )
```

Appendix B. Dashboard screenshots

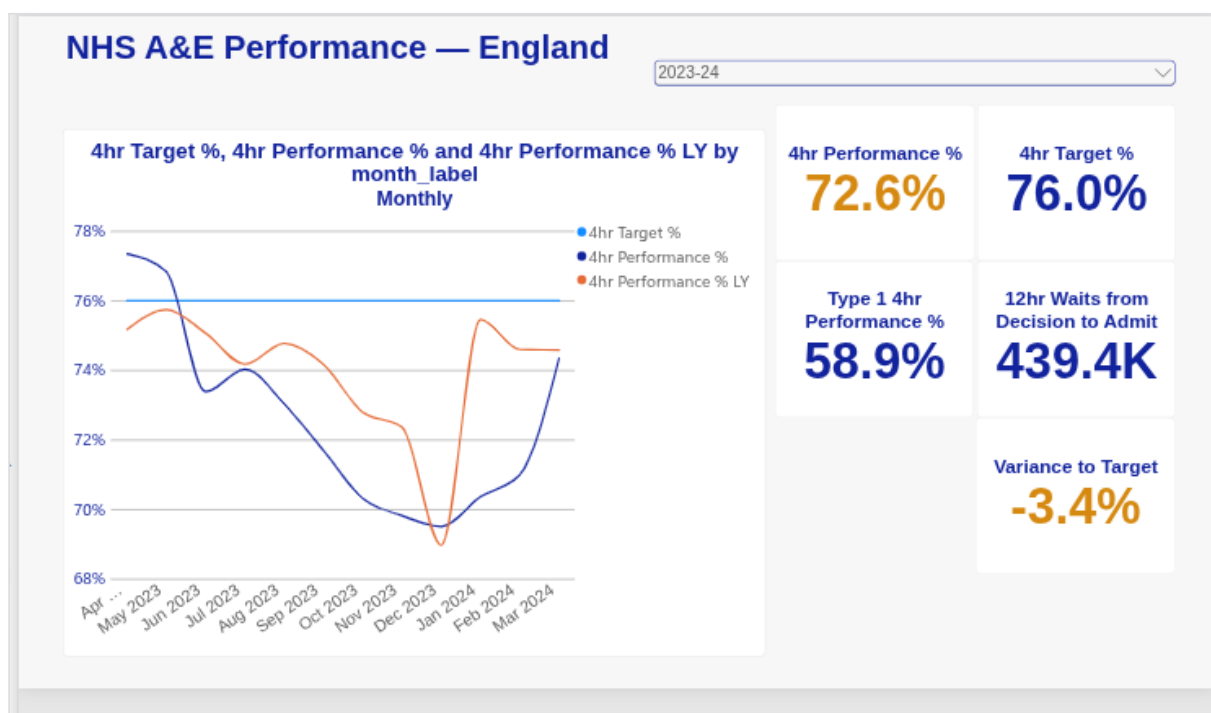


Figure 5. National Overview page (2023-24 selected): headline KPIs with RAG colours, and 4-hour performance this year vs last year vs target.

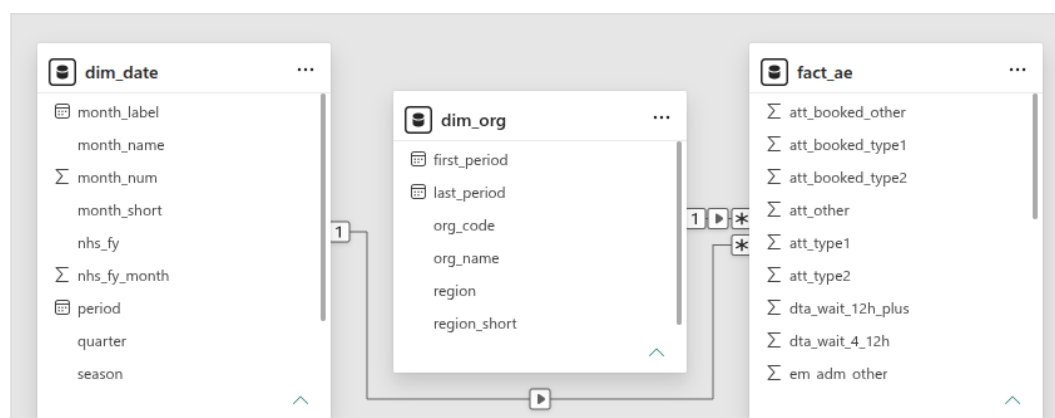


Figure 6. Semantic model: a star schema with fact_ae linked to dim_org and dim_date (many-to-one, single-direction filtering).

Sources

- NHS England, A&E Attendances and Emergency Admissions statistics: england.nhs.uk/statistics/statistical-work-areas/ae-waiting-times-and-activity/
- NHS England statistical commentary, August and September 2026 (validation figures).
- NHS.uk, Royal Albert Edward Infirmary: Accident and emergency services (24-hour Emergency Department).
- Greater Manchester Integrated Care: Urgent treatment centres in Wigan borough (Wigan UTC and Leigh UTC).
- Health Innovation Manchester: Wrightington, Wigan and Leigh NHS Foundation Trust (sites and services).
- Project repository: github.com/dynamixAI/nhs-ae-performance-powerbi

Appendix C. Glossary

Plain-English explanations of the organisations, column headings and terms used in this report.

Organisations and services

Term	Meaning
WWL	Wrightington, Wigan and Leigh Teaching Hospitals NHS Foundation Trust: the NHS organisation that runs hospital services across the Wigan borough. Its code in the data is RRF.
NHS trust	An organisation that runs one or more hospitals or health services in an area. NHS England publishes A&E figures per trust, not per hospital building.
Wigan Infirmary (RAEI)	The Royal Albert Edward Infirmary in Wigan: WWL's main hospital and the home of its 24-hour Emergency Department and the Wigan Urgent Treatment Centre.
Major A&E (Type 1)	A full, consultant-led Emergency Department open 24 hours, for serious injuries and life-threatening emergencies. WWL's is at Wigan Infirmary.
Urgent treatment centre (UTC)	A walk-in service for minor injuries and illnesses, such as sprains, cuts and minor infections. WWL runs two: at Wigan Infirmary and at Leigh Health Centre. Reported as "Other" A&E types in the data.
North West	One of seven NHS England regions. It includes Greater Manchester, Lancashire, Merseyside and Cheshire. Used here as WWL's regional benchmark.

Column headings and measures

Term	Meaning
Attendances / visits	The number of times people came to A&E. One person who visits twice counts as two attendances.
4hr % (overall, headline)	The percentage of all A&E patients, in both the major A&E and urgent treatment centres, who were admitted, transferred or discharged within 4 hours of arriving. Example: 70% means 7 in every 10 patients.
Type 1 4hr %	The same measure for the major A&E only. Usually lower than the headline, because major A&Es treat the sickest, most complex patients.
UTC 4hr %	The same measure for the urgent treatment centres only.
Target	The 4-hour level the NHS asked trusts to reach in each financial year: 76% (2023-24), 78% (2024-25 and 2025-26) and 82% (2026-27). No interim target existed before 2023-24.
vs Target (variance)	How far performance is above or below the target, in percentage points. Example: 70.6% against a 76% target is -5.4 points.
RAG status	A traffic-light rating. Green: at or above target. Amber: up to 5 points below. Red: more than 5 points below.
Decision to admit (DTA)	The moment A&E doctors decide a patient needs to stay in hospital and be moved to a ward.
12hr bed waits	Patients who waited more than 12 hours for a hospital bed after the decision to admit, often called "trolley waits". A key sign that wards are full.
12hr waits per 1,000 visits	12-hour bed waits divided by major A&E visits, multiplied by 1,000. This makes large and small trusts fair to compare.
Emergency admissions	Patients admitted to hospital in an emergency, either through A&E or by another route, such as a GP referral straight to a ward.
Booked appointments	A&E patients given a booked time slot instead of waiting in the queue. They are included in all 4-hour measures, as in NHS England's method.

Analysis terms

Term	Meaning
Percentage points (pts)	The difference between two percentages. 75% to 82% is a gap of 7 percentage points, not 7 percent.
NHS financial year	April to March, written like 2023-24. NHS performance and targets are set for these years rather than calendar years.
Partial year (*)	A year with only some months of data: 2020-21 (January–March 2021) and 2026-27 (April–September 2026). Not directly comparable with full years.
Benchmark	A comparison group used to judge whether a result is good or bad, here the North West and England.
Like-for-like comparison	Comparing the same months in different years, for example April–September 2025 vs April–September 2026, so seasonal effects such as winter cannot distort the result.
Mix effect	When an average changes because the blend of what it contains changes, not because any part improved. WWL's headline rose partly because more patients used the fast urgent treatment centres.
Exit block	When patients cannot leave A&E because no ward bed is free, so A&E fills up and everyone waits longer.
Case mix	The type and seriousness of patients a service treats. Sicker patients take longer.
Outlier	A result that is very different from similar cases, and so worth investigating.
Reconciliation	Checking cleaned data adds up to the source's own published totals, to prove nothing was lost or double-counted.

Tools and data terms

Term	Meaning
Power BI	Microsoft's business intelligence tool, used here to build the data model and dashboard.
DAX	Data Analysis Expressions: the formula language used in Power BI to define measures such as 4hr %.
Measure	A calculation that recalculates automatically for whatever is selected, such as one trust, one region or one year.
Star schema	A data model with one central table of numbers (the fact table) linked to descriptive tables (dimensions) for organisations and dates.
Open Government Licence	The licence NHS England data is published under, which allows reuse with acknowledgement of the source.